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how long does orthotist spend the time to write AT report for NDIS

For an orthotist preparing an NDIS Assistive Technology (AT) report, the time spent can vary quite a bit depending on complexity, but in real clinical practice there are fairly consistent ranges.

Typical time breakdown

1. Simple / Low-cost AT (e.g. basic AFO under \$1,500)

30–60 minutes

Often shorter justification

May reuse template language

Minimal clinical reasoning required

2. Standard AT report (most orthotic cases)

1.5 – 3 hours total

This is the most common range

Includes:

Clinical assessment notes review

Functional impact description

Justification linked to NDIS criteria

Risk of non-provision

Quotes and specifications

3. Complex AT / High-cost (e.g. KAFO, carbon devices, neurological conditions like MS, ALS, CP)

3 – 6+ hours

More detailed sections required:

Progressive condition justification

Trial outcomes or comparative options

Multidisciplinary input (physio, OT)

Evidence-based rationale

What actually takes the time

The writing itself is only part of it. Time is usually spent on:

Translating clinical findings ÷ NDIS language

Aligning with NDIS “reasonable and necessary” criteria

Linking impairments ÷ functional limitations ÷ participation impact

Customising (templates help, but can't fully automate for complex cases)

Real-world note (especially in Sydney clinics)

Many orthotists:

Block 2–3 hours per standard report in their schedule Or split it: 1 hr

drafting 30–

60 min editing

+ evidence + quoting Quick summary Basic: 0.5–1

hr

Standard: 1.5–3 hrs (most common)

Complex: 3–6+ hrs

break this down into billable vs non-billable time for NDIS, or help

you structure a faster template workflow (cuts report time by ~30–50% in practice).

do both

Alright—here's both the NDIS billing side and a practical workflow to cut your report time significantly, based on how orthotists actually run this in clinics.

1. NDIS Billing: What time can you actually charge?

Under the NDIS framework, AT report writing usually falls under:

• Capacity Building – Improved Daily Living

(Line item typically used by orthotists)

What is generally billable

You can legitimately bill for:

Clinical assessment (face-to-face or telehealth)

AT assessment & trials

Report writing (clinical justification)

Liaising with other clinicians (OT, physio, GP)

Reviewing outcomes / follow-up documentation

In practice:

Report writing IS billable, as long as it is clinically necessary and documented •

What often becomes non-billable (or risky to claim)

Excessive admin (copy-paste / formatting)

Rewriting due to poor initial documentation

Long back-and-forth emails not tied to clinical reasoning

Rejected reports requiring full rewrite (sometimes disputed)

Typical billing pattern (real-world)

For a standard orthotic AT report:

Assessment: 1–2 hrs

Report writing: 1.5–3 hrs

Liaising: 0.5–1 hr

• Total billed: ~3–5 hours

For complex cases (ALS, MS, CP, KAFO):

Total can go up to 5–8+ hours

Key compliance tip

NDIS planners look for:

Clear link: impairment • functional limitation • participation restriction

Why this AT (and not cheaper alternatives)

Risks if not funded

If those are weak • report gets rejected • more unpaid time

• 2. How to cut report time by 30–50% (practical workflow)

This is where most orthotists lose time. Templates alone don't fix it—you need a structured system.
Machine Translated by Google

Step-by-step efficient workflow

1. Capture “NDIS-ready” notes during assessment (biggest time saver)

Instead of writing:

“Foot drop, reduced DF”

Write directly in NDIS language:

“Left foot drop resulting in reduced toe clearance, increasing falls risk and limiting safe community ambulation”

This alone can cut 30–40% of writing time

2. Use a modular template (not one big document)

Break your report into reusable blocks:

Condition summary (e.g. MS / CP / ALS variants)

Functional impact paragraphs

Orthotic justification (AFO / KAFO / carbon)

Risk of non-provision

Alternatives trialled

Then you just select + tweak, not rewrite

3. Pre-write common justification banks

Example:

Carbon AFO justification (reusable core):

Energy return ÿ improves gait efficiency

Reduces fatigue

Enables longer community participation

Then customise 20–30% only

4. Use a “3-layer writing method”

Instead of writing perfectly in one go:

Layer 1 (10–15 min): dump key points

Layer 2 (30–60 min): structure into sections

Layer 3 (20–30 min): polish for NDIS language

Faster than trying to write perfectly from the start

5. Standardise objective phrases

Have ready-to-use lines like:

“Without this device, the participant is at increased risk of falls and injury”

“This limitation significantly impacts independent mobility and community access”

“A lower-cost alternative is not suitable due to...”

Saves surprising amounts of time

Save common device specs (AFO, KAFO, BSAFO, carbon)

Pre-fill supplier details

7. Final 10-minute compliance check (prevents rework)

Before submitting, quickly confirm:

• Is it clearly “reasonable & necessary”?

• Have you ruled out cheaper options?

• Is functional impact strong enough?

• Is risk clearly stated?

This step prevents hours of unpaid rewrites

What this looks like in reality

Scenario Without system With system Standard

AFO report 2.5–3 hrs 1.5–2 hrs Complex neuro case

4–6 hrs 2.5–4 hrs Bottom line

Yes, report writing is billable under NDIS—but only when clearly clinically justified

Most inefficiency comes from:

Poor initial notes

Rewriting from scratch

Weak NDIS language causing rejection

Fix those, and you:

Save 1–2+ hours per report

Reduce rejection risk

Improve approval rate

If you want, I can build you a

plug-and-play orthotist AT report template (NDIS-optimised) or even a

condition-specific version (MS, CP, ALS) that matches what planners actually approve.